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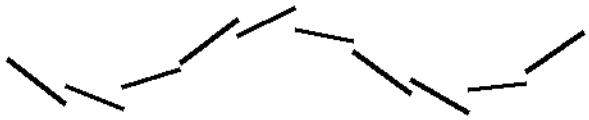

The Psychological Effects of Social Isolation During COVID-19

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Abstract

This longitudinal study tracked 218 adults over 24 months measuring depression, anxiety, loneliness, and cognitive function at six-month intervals. Results indicate significant deterioration in all domains during the first six months of lockdown, with partial recovery thereafter. Participants who maintained virtual social contact recovered significantly faster on loneliness and depression measures.

1. Introduction

Social isolation is among the most well-documented risk factors for poor mental health outcomes. The pandemic created an unprecedented natural experiment in which isolation was imposed broadly across age groups and occupations simultaneously. Early studies confirmed acute increases in depression and anxiety. Fewer studies tracked the same individuals longitudinally to document whether effects persisted, attenuated, or worsened as restrictions were periodically relaxed and reimposed.

2. Methods

Participants were recruited in March 2020, yielding 218 adults who completed all five assessment waves through March 2022. At each wave, participants completed the PHQ-9 for depression, GAD-7 for anxiety, UCLA Loneliness Scale, and the Montreal Cognitive Assessment.

3. Results

Depression scores peaked at wave 2 with a mean of 11.4, up from baseline of 6.2. Participants reporting high virtual social contact showed PHQ-9 of 7.1 at wave 5 compared to 11.3 for low virtual contact. Cognitive function declined from 24.8 to 22.1 by wave 3 and did not recover, remaining at 22.3 by the final wave.

4. Discussion and Conclusion

The protective effect of virtual social contact aligns with attachment theory. The cognitive findings are concerning and may reflect reduced stimulation, inactivity, and chronic stress. Public health planning for future crises should incorporate mental health support infrastructure alongside physical health resources.

References

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